INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Vinorelbine intravenous (IV) · advanced breast cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar	Treatment stages	✓ Regimen source
21 days	**1**	**NCCP 00232**
See the active treatment phase	Explained in the treatment reference	Version 5

21-day cycle

Treatment stages

01 **Vinorelbine IV · advanced breast cancer**
21-day reference

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	9
04	Sources and further information	14

My name

Hospital / oncology team

☎ Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCheck v0.84.45

Feasibility build. Clinical, pharmacy and patient/PPI review pending.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Use the treating unit's 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

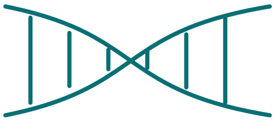
How the medicines work

Aim to slow or shrink advanced breast cancer and help symptoms while balancing side effects. Duration depends on benefit, tolerability and the agreed plan; a fixed number of cycles or a guaranteed survival improvement is not promised.

VINCA ALKALOID CHEMOTHERAPY

Vinorelbine

IV infusion



Disrupts the machinery of cell division

Vinorelbine disrupts the tiny structures a cell needs to divide. This can stop cancer cells multiplying. Healthy blood-forming cells, gut cells and nerves can also be affected.

Binds cell-division structures → Microtubules cannot function normally → Cell division is disrupted

What to watch for

Use the treating unit's 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The treatment being discussed

Breast treatment context

Advanced breast cancer. This patient suite describes the breast setting; the source protocol also has other tumour indications with their own clinical context.

Schedule and duration

30 mg/m² onDays 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/ unacceptable toxicity; team review determines continuation.

How treatment is given

NCCP specifies 50 mL sodium chloride 0.9% over 15 minutes followed by 250 mL saline. Current Navelbine SmPC says 6–10 minutes: product/local-pharmacy reconciliation is required. Do not promise a total visit length; preparation, tests and observation add time.

Why this treatment may be chosen

Vinorelbine has shown activity in advanced breast cancer. Expected benefit depends on cancer biology, earlier treatment and health; historical trial averages do not predict an individual result.

The team considers previous treatment, cancer features, health and your priorities. A response in a trial cannot guarantee benefit for an individual.

CHAPTER 02

Your schedule, tests and appointments

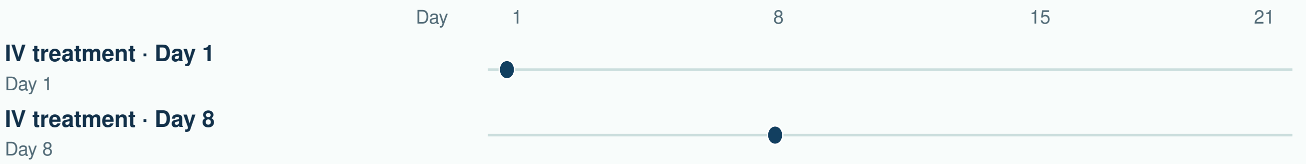
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

The phases and how they connect

Vinorelbine IV · advanced breast cancer

21-day view



IV treatment · Day 1
Day 1

IV treatment · Day 8
Day 8

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY REFERENCE

Vinorelbine IV · advanced breast cancer

21-day interval

30 mg/m² onDays 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/ unacceptable toxicity; team review determines continuation.

These are the medicines in this exact schedule. Dose holds, missed tablets and omitted visits need instructions from your treating team.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones are not equal time intervals. Your team books and reviews tests. Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00232 v5 · official regimen · Current Irish product information · safety supplement

THE WHOLE TREATMENT JOURNEY

Vinorelbine IV · advanced breast cancer: treatment and checks

Advanced breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00232	Confirm breast plan and current dose	Days 1 and 8 each 21-day cycle	Continue while helpful and tolerated	Review dose and recovery before restart	Agree continuation / next treatment

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Blood / kidney / liver NCCP 00232 · SMPC	Baseline blood and organ tests	Day 1 and Day 8 blood counts	Counts before every dose; organ tests each cycle	Review recovery and current dose	Follow unresolved abnormalities
Symptoms / medicines NCCP 00232 · SMPC	Bowel / nerve / infection review	Report early symptoms and support problems	Review all medicines and side effects	Do not restart or escalate yourself	Agree continuation and response assessment
Special schedule check NCCP 00232 · SMPC	Confirm Day 1 and Day 8 visits	Check Day 8 clearance	Continue 21-day pattern	Keep previous dose / nadir history	Personal plan; no booked dates implied

What each check is for

1

Treatment and dose review

Before every administration

Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

Ask / act: The treating team confirms results, dose and suitability; this map does not authorise treatment.

2 Support and symptoms

Throughout treatment, including days at home

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Ask / act: Keep a written medicine plan and urgent-contact route.

3 Response and continuation

At agreed clinical review points

Aim to slow or shrink advanced breast cancer and help symptoms while balancing side effects. Duration depends on benefit, tolerability and the agreed plan; a fixed number of cycles or a guaranteed survival improvement is not promised.

Ask / act: Discuss benefit, toxicity and priorities before continuing or changing treatment.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Visits and tests

Two infusion visits per 21-day cycle, with blood tests before doses. Peripheral cannula or existing access according to the treating service; a port is not automatically required.

STEP 2

Treatment preparation

Wear clothing allowing IV-site observation; report discomfort immediately.

STEP 3

At home

No home vinorelbine capsules are part of this regimen. Keep an up-to-date medication list and emergency contact.

Checks before and during treatment

Every dose depends on the reviewed treatment plan and results.

Blood and symptom checks

Heart and response checks

Review cardiovascular history and new symptoms. These monotherapy NCCP protocols do not mandate the serial left ventricular ejection fraction (heart pumping measurement) (LVEF)/echo schedule used for anthracyclines or trastuzumab; arrange cardiac tests when clinically indicated.

Fertility, pregnancy and breastfeeding

Purpose and alternatives for the person’s advanced breast cancer, including other systemic treatment or supportive care according to goals. Possible benefit is uncertain; response is reviewed and treatment can be paused, changed or stopped. Low blood counts, infection/sepsis, fatigue, nausea/vomiting, bowel problems, neuropathy, hair change and uncommon serious lung/heart/allergic toxicity. Fertility and pregnancy planning before treatment; discuss sperm/egg preservation where relevant. Do not breastfeed. HPRA supplements: contraception during treatment and for 7 months afterwards for women,4 months for men; confirm the exact product and individual advice.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Vinorelbine IV · advanced breast cancer

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
IV treatment · Day 1	**Day 1** Only after team clearance
IV treatment · Day 8	**Day 8** Only after team clearance
Blood counts / infection	**Days 1–21** Fever or sudden illness needs urgent help on any day
Nausea / bowel symptoms	**Days 1–21** Use the prescribed support plan
Nerve / fatigue symptoms	**Days 1–21** Report new or worsening symptoms early

Treatment and recovery: 30 mg/m² onDays 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

Tests and decisions: Check blood count, kidney and liver function before each cycle, and blood counts on the day of every administration including Day 8. Review neuropathy, bowel symptoms and infection each cycle; abnormal results may postpone or change treatment.

> **Why these bands stay broad:** This is a relative reference, not an appointment calendar. Treatment days can change after clinical review. Symptom bands do not predict when an effect will occur. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00232 v5 · official regimen ↗

> **There are no guaranteed “safe days”.** A day without treatment is not automatically a safe day. Use the emergency contact when needed; do not move or restart doses yourself.

Between-treatment notes

Day 1–21: One reference cycle

30 mg/m² onDays 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation. Follow the written plan from your treatment team; no dates are entered here.

Support around the medicines

1

Sickness plan

No routine preventive anti-sickness medicine is usually required for this IV regimen. Ask for a rescue plan and report uncontrolled symptoms.

2

Bowel care

Agree a constipation plan, particularly if using opioids or ondansetron. Severe abdominal pain, swelling, repeated vomiting or inability to pass stool/gas needs urgent assessment; do not simply add laxatives.

3

Medicines and vaccines

Check every new prescription, supplement and vaccine with the team; avoid grapefruit. Live vaccines and interacting medicines need review.

Support and safety at home

Keep the written medicine and urgent-contact plan.

Medicines and vaccines

Questions worth asking

Can IV and oral vinorelbine be exchanged?

For IV use only; intrathecal administration may be fatal. Oral and IV vinorelbine are not interchangeablemg-for-mg. Stop infusion and tell staff immediately about pain, burning, swelling or leakage at the IV site; extravasation can damage tissue.

How long is an infusion visit?

The source describes a short infusion followed by a saline flush. Your unit confirms the product-specific infusion instructions and complete visit length, including blood results, preparation and observation.

> **Try explaining the plan back:** Which treatment days and doses are prescribed, which blood tests must happen first, how do you take the support medicines, and which symptoms need urgent help?

Vinorelbine

- Low blood counts, infection and bleeding
- Nausea, vomiting and fatigue
- Constipation or bowel obstruction
- Nerve effects and uncommon lung/heart toxicity
- Infusion-site injury

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Vinorelbine IV · reviewed support plan

Active medicines: Vinorelbine

Emetogenic risk: Minimal IV risk

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Individual antiemetic plan differs by route; list routine and rescue medicines separately.
- Discuss constipation prevention, hydration/diet where appropriate and prescribed laxatives; regular opioid use increases the need for a bowel plan.
- No routine steroid premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated by these NCCP monotherapy documents. Consider additional prevention based on individual risk/local policy; do not create a generic automatic prescription.
- Gastric protection is a consideration for high gastrointestinal-bleed risk, not a universal drug.
- Confirm strong CYP3 A4 inhibitors/inducers, P-gp modulators, phenytoin, itraconazole, warfarin and live vaccines. Yellow fever vaccine is contraindicated. Avoid grapefruit per NCCP; do not self-start supplements.
- No routine prophylaxis required by NCCP minimal-risk pathway.
- Prescriber selects an as-needed medicine according to symptoms, constipation, QT/interactions and local policy. Reassess before future doses if nausea/vomiting occurs.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: ___________________________________
Prescriber / date: __________________ Pharmacy review: __________________

NATIONAL CANCER CONTROL PROGRAMME (NCCP) 00232 · VERSION 5

Vinorelbine intravenous (IV) · advanced breast cancer

30 mg/m² onDays 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Blood counts and infection

Vinorelbine

White blood cells can fall, raising the risk of infection. Anaemia can cause tiredness or breathlessness; low platelets can cause bruising or bleeding. Blood tests guide delays and dose changes. Fever, shaking chills or feeling suddenly unwell need the oncology emergency contact immediately.

3 Tingling, numbness or weakness

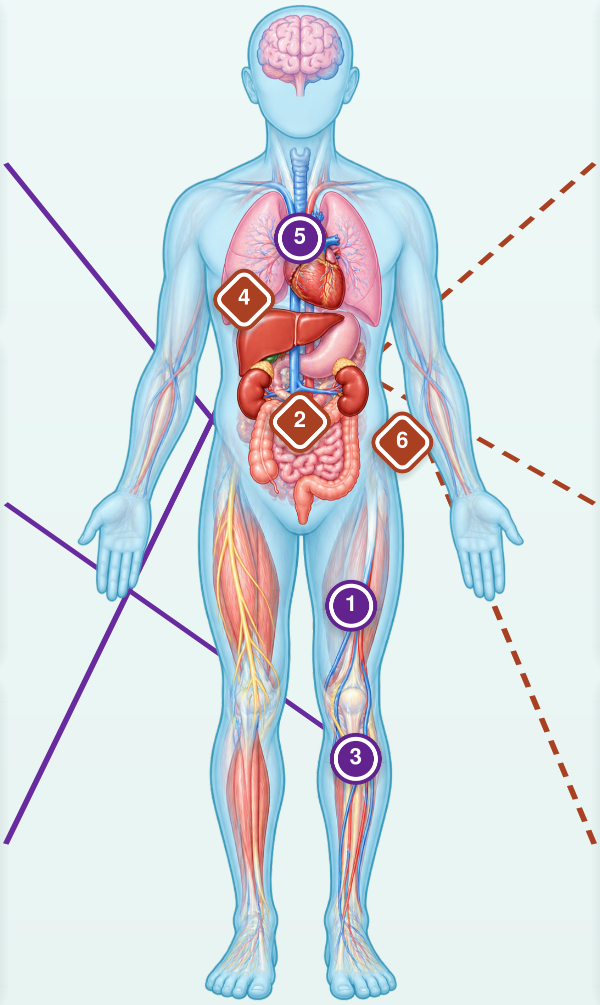
Vinorelbine

Nerve effects may change sensation, grip, walking or bowel movement. Report changes early because the next dose may need to wait or be reduced.

5 Uncommon serious symptoms

Vinorelbine

New chest pain, sudden or worsening breathlessness, or severe allergic symptoms need urgent medical help. Previous heart disease and interacting medicines need review before treatment.



2 Constipation, nausea and mouth problems

Vinorelbine

The bowel can slow down and nausea, vomiting, diarrhoea or sore mouth can occur. Use the prescribed bowel and anti-sickness plan. Severe abdominal pain, swelling, repeated vomiting or inability to pass stool/gas needs urgent assessment for ileus; do not simply take extra laxatives.

4 Fatigue, hair and liver tests

Vinorelbine

Tiredness, hair thinning/loss and liver-test changes can occur. Pace activities and tell the team if eating, drinking or daily tasks become difficult.

6 Infusion site

Vinorelbine

Tell staff immediately about burning, pain, swelling or leakage.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects are shown. Symptoms can occur on any day; this is not a complete list.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00232 v5 · official regimen**
   https://healthservice.hse.ie/documents/6567/232_v5_Vinorelbine_IV.pdf
2. **Current Irish product information · safety supplement**
   https://assets.hpra.ie/products/Human/26035/Licence_PA0329-011-003_08012024110935.pdf
3. **NCCP antiemetic guidance v6 · minimal IV pathway**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
4. **NCCP NCIS antiemetic medicines v8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **Martín M et al. Lancet Oncology. 2007;8:219–225. DOI 10.1016/S1470-2045(07)70041-4**
   https://www.sciencedirect.com/science/article/pii/S1470204507700414